

Chest Pain (HEART Score) - Female (Adult)

Algorithm 1042 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only adult chest-pain pathway that sends active or suspected acute coronary syndrome symptoms directly to Qatar 999 without HEART scoring delay. HEART may be considered only after documented in-person clinician assessment, 12-lead ECG and troponin; it must never be calculated or implied by telephone or used as a standalone rule-out test.
- Adult patient.
- Female.

Clinical background

- HEART is an emergency-department risk-stratification aid for adults with chest pain being assessed for possible acute coronary syndrome. It combines clinician-assessed history, 12-lead ECG, age, risk factors and measured troponin. A telephone call cannot supply the ECG, troponin or complete clinician assessment, so individual component points and a total score must not be displayed or used for disposition. It is not a diagnostic test and is not validated for children or for screening people without a suspected acute coronary syndrome presentation.

Pain severity

- Mild (1-3): Does not interfere with normal activity, movement, concentration, or sleep.
- Moderate (4-7): Interferes with normal activity, movement, concentration, or sleep.
- Severe (8-10): Prevents normal activity or movement, or is described as excruciating.

Initial assessment

1. [OPEN_TEXT] Describe the chest pain in your own words - what does it feel like, where is it, does it spread anywhere?
2. [DURATION] When did the pain start, and how long has it lasted?
3. [YES_NO] Is the pain present now, recurring or worsening; accompanied by breathlessness, sweating, nausea, faintness, weakness, or pain in the arm, shoulder, back, neck or jaw; or otherwise suspected by the nurse to represent acute coronary syndrome? If yes, stop assessment and use the Qatar 999 emergency branch without calculating HEART.
4. [YES_NO] Is the patient under 18, pregnant or recently postpartum, or do they have known heart or vascular disease, diabetes, smoking, high blood pressure or cholesterol?
5. [PAIN_SEVERITY] What is the current pain severity from 0 to 10, and how is it affecting normal activity or movement?
6. [PREGNANCY] Is the patient pregnant or possibly pregnant? If yes, record gestation where known.

Triage and redirect questions

1. Redirect → Chest Injury

Did the pain begin immediately after a direct blow, fall, or other chest injury, with the injury as the main concern and none of the cardiac emergency features below?

A clearly traumatic chest complaint belongs in the chest-injury pathway. Any pressure-like, radiating, exertional, breathless, sweaty, faint, or persistent pain stays in this pathway for cardiac assessment.

1. Disposition level 100

Is chest pain or pressure present now, severe, recurring or worsening, or accompanied by shortness of breath, sweating, nausea, fainting/near-fainting, marked weakness, or pain spreading to the arm, shoulder, back, neck or jaw?

This is an emergency symptom screen, not part of HEART. Acute coronary syndrome can be atypical, including in women, older adults and people with diabetes; a reassuring telephone description cannot exclude it.

1. Disposition level 100

Even if the symptom description is atypical or incomplete, does the nurse suspect active or recent acute coronary syndrome, or remain unable to exclude it safely during this call?

Suspected acute coronary syndrome requires Qatar 999 emergency assessment without scoring delay. HEART applies only after in-person clinician assessment with a documented 12-lead ECG and measured troponin; it is not a telephone rule-out or standalone triage tool.

2. Disposition level 70

Only after the emergency and suspected-acute-coronary-syndrome branches are negative: has the chest pain resolved, but this episode has not yet had an in-person clinician assessment with a documented ECG and troponin testing?

Resolved symptoms still need prompt in-person assessment, and any remaining suspicion of acute coronary syndrome belongs in the preceding Qatar 999 branch. Age is collected for the receiving clinician but is not scored by the telephone pathway.

3. Disposition level 70

Is the patient under 18, pregnant/recently postpartum, or is another serious cause possible (for example aortic, pulmonary embolic, respiratory or traumatic chest pain)?

HEART is not a pediatric rule and must not substitute for evaluation of pregnancy-related or non-coronary emergencies. Route according to symptoms and in-person clinical assessment, never a telephone score.

4. Disposition level 70

Regardless of age, risk factors or how non-cardiac the description seems, is a complete in-person assessment for this episode unavailable?

A HEART total cannot be calculated without all five components in the intended clinical setting. No telephone low-risk or self-care tier exists.

Advice

Emergency chest pain precautions

- Call Qatar emergency services on 999 now. Keep the person resting, do not let them drive, and follow the emergency call-taker's instructions. Do not recommend aspirin, glyceryl trinitrate or other medicine unless specifically directed under an approved Qatar protocol or already prescribed for this exact situation. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening pain; loss of consciousness; difficulty breathing.

Chest pain requires in-person evaluation

- Arrange prompt in-person assessment at a Qatar service capable of clinical examination, a documented 12-lead ECG and measured troponin. Do not calculate, display or interpret HEART before all required components are obtained in person, and do not use HEART alone to rule out acute coronary syndrome. Do not self-drive if symptoms recur or the person feels unwell; call 999 if any emergency feature appears or acute

coronary syndrome is suspected. The exact non-emergency destination, troponin strategy and transport pathway are GOVERNANCE_REQUIRED.

- Call Back If: pain worsens or spreads; shortness of breath develops; sweating or nausea develops.

First aid

- Call Qatar emergency services on 999 now. Keep the person resting, do not let them drive, and follow the emergency call-taker's instructions. Do not recommend aspirin, glyceryl trinitrate or other medicine unless specifically directed under an approved Qatar protocol or already prescribed for this exact situation. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Six AJ, Backus BE, Kelder JC. Chest pain in the emergency room: value of the HEART score. Neth Heart J. 2008;16(6):191-196.
- Backus BE, Six AJ, Kelder JC, et al. A prospective validation of the HEART score for chest pain patients at the emergency department. Int J Cardiol. 2013;168(3):2153-2158.
- Gulati M, Levy PD, Mukherjee D, et al. 2021 AHA/ACC Chest Pain Guideline. Circulation. 2021;144:e368-e454.

Provenance

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